

C812

Healthcare Reimbursement

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 320-bed hospital that recently opened a new cancer wing treats a Medicare inpatient assigned to an MS-DRG with a relative weight of 1.8. The hospital's base rate is \$6,500. The patient stayed 9 days, longer than the average of 5, and did not qualify for an outlier payment. What is the expected MS-DRG payment?

- A. \$21,060
- B. \$6,500
- C. \$3,611
- D. \$11,700

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

2. [TRANSFER — reversed stem / different variable]

A hospital with a base rate of \$5,000 received \$14,000 for a Medicare inpatient case with no outlier. What was the MS-DRG relative weight?

- A. 1.0, because the base rate already reflects the case
- B. 2.8
- C. 0.36
- D. \$9,000

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

3.

A hospital-owned outpatient surgery department in a suburb with fast population growth performs a knee arthroscopy on a Medicare patient, who goes home the same day. The department recently hired a new revenue cycle director. Under which system is the hospital's facility fee paid?

- A. Billed charges, because prospective payment never applies to same-day care.
- B. OPPS, using APCs.
- C. IPPS, using MS-DRGs.
- D. The Physician Fee Schedule (RBRVS).

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

4.

A physician in a multispecialty group, which has been in practice since 1985, performs a service with 3.0 total RVUs after geographic adjustment. The conversion factor is \$33.00. The group bills this service about 400 times a year. What is the Medicare fee schedule amount for one service?

- A. \$33.00
- B. \$11.00
- C. \$99.00
- D. \$39,600

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A clinic with a new patient portal and a large Medicaid population finds that 12% of its claims are denied as “patient not eligible on date of service.” The billing office has five experienced staff. Where is the best fix?

- A. Verify insurance eligibility at scheduling and again at registration.
- B. Hire more staff to appeal the denials.
- C. Write them off, since eligibility denials can never be prevented.
- D. Improve coding accuracy.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A hospital, which recently rebranded after a merger, launched a new cardiac monitoring service in July, but the service was never added to the chargemaster. Nurses documented every session carefully. What is the most likely revenue cycle result?

- A. Overbilling, because the rebrand changed prices.
- B. Lost charges: the sessions are not billed.
- C. Claims will be denied for lack of medical necessity.
- D. No effect, because documentation automatically creates the charge.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A physician practice with 14 providers and a new billing vendor has \$1.2 million in net accounts receivable. Its net patient revenue for the past 365 days was \$10.95 million. What is days in accounts receivable?

- A. 0.11
- B. 9.1 days
- C. 40 days
- D. 365 days

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

8. [TRANSFER — reversed stem / different variable]

A hospital's goal is 45 days in A/R. Its average daily net patient revenue is \$200,000. What is the highest net A/R balance that meets the goal?

- A. \$9.0 million
- B. \$73 million
- C. \$4,444
- D. \$0, because any balance above zero misses the goal

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

9.

A 67-year-old engineer who still works full-time at a company with 300 employees has Medicare Part A and coverage through his employer's group health plan. He recently completed a marathon. He is admitted for a hip repair. Which payer is primary?

- A. Each plan pays half.
- B. Medicare, because inpatient stays fall under Part A.
- C. Medicare, because he is over 65.
- D. The employer group health plan.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

10. [TRANSFER — reversed stem / different variable]

A 72-year-old retiree has Medicare and a retiree health plan from her former employer. Which pays first?

- A.** Medicare.
- B.** The retiree plan.
- C.** Whichever plan has the larger deductible.
- D.** Employer-sponsored coverage always pays first.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A primary care group that recently moved to a larger building accepts a capitated contract covering 2,000 members at \$30 per member per month. Last year its physicians averaged 22 patient visits per day. What is the group's annual revenue from this contract?

- A.** It depends on how many visits members make.
- B.** \$60,000
- C.** \$720,000
- D.** \$66.67 per member

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

Under a capitated contract, a provider group reduces unnecessary follow-up visits by 10% while membership stays the same. What happens to its capitation revenue?

- A.** It falls 10%.
- B.** It rises 10%.
- C.** It falls, because capitation always penalizes lower use.
- D.** It stays the same, and lower costs improve the margin.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 320-bed hospital ~~that recently opened a new cancer wing~~ treats a Medicare inpatient assigned to an MS-DRG with a relative weight of 1.8. The hospital's base rate is \$6,500. ~~The patient stayed 9 days, longer than the average of 5,~~ and did not qualify for an outlier payment. What is the expected MS-DRG payment?

Target word: DRG payment **Decides:** Relative weight $\times$ base rate; no outlier.

A	Camouflage	Scales the payment by the 9 / 5 day ratio; DRG payment is fixed per case.
B	Dropped element	Leaves out the relative weight.
C	Inverted operation	Divides the base rate by the weight.
D	Correct	$1.8 \times \$6,500 = \$11,700$, regardless of the 9-day stay.

2. Answer B Concept 1 • transfer

A hospital with a base rate of \$5,000 received \$14,000 for a Medicare inpatient case with no outlier. What was the MS-DRG relative weight?

Target word: solve for relative weight **Decides:** Weight = payment / base rate.

A	Dropped element	Ignores that payment differs from the base rate.
B	Correct	$\$14,000 / \$5,000 = 2.8$.
C	Inverted operation	Divides the base rate by the payment.
D	Confusion set	Subtracts instead of divides; a weight is a ratio, not dollars.

3. Answer B Concept 2

A hospital-owned outpatient surgery department ~~in a suburb with fast population growth~~ performs a knee arthroscopy on a Medicare patient, who goes home the same day. ~~The department recently hired a new revenue cycle director.~~ Under which system is the hospital's facility fee paid?

Target word: payment system for hospital outpatient facility fee **Decides:** Hospital outpatient setting, facility claim.

A	Absolute language	Outpatient care has its own prospective system.
B	Correct	Medicare pays hospital outpatient facility services under OPPTS/APCs.
C	Confusion set	MS-DRGs pay inpatient stays.
D	Adjacent category	That pays the surgeon's professional service, not the hospital facility fee.

4. Answer C Concept 3

A physician in a multispecialty group, ~~which has been in practice since 1985,~~ performs a service with 3.0 total RVUs after geographic adjustment. The conversion factor is \$33.00. ~~The group bills this service about 400 times a year.~~ What is the Medicare fee schedule amount for one service?

Target word: fee for one service **Decides:** RVUs $\times$ conversion factor.

A	Dropped element	Leaves out the RVUs.
B	Inverted operation	Divides the conversion factor by the RVUs.
C	Correct	$3.0 \times \$33.00 = \99.00 .
D	Camouflage	Multiplies by annual volume; the question asks for one service.

5. Answer A Concept 4

A clinic ~~with a new patient portal~~ and a large Medicaid population finds that 12% of its claims are denied as “patient not eligible on date of service.” ~~The billing office has five experienced staff.~~ Where is the best fix?

Target word: root cause location **Decides:** Eligibility denials start at scheduling and registration.

A	Correct	Front-end eligibility checks prevent this denial type.
B	Inverted operation	Works the problem from the back end after the fact.
C	Absolute language	They are among the most preventable denials.
D	Adjacent category	Coding does not cause eligibility denials.

6. Answer B Concept 5

A hospital, ~~which recently rebranded after a merger,~~ launched a new cardiac monitoring service in July, but the service was never added to the chargemaster. ~~Nurses documented every session carefully.~~ What is the most likely revenue cycle result?

Target word: effect of a missing chargemaster line **Decides:** No chargemaster entry means no charge is generated.

A	Camouflage	The rebrand is irrelevant to the missing line.
B	Correct	Without a chargemaster line, charge capture fails.
C	Adjacent category	Necessity denials require a claim; here no charge reaches a claim.
D	Dropped element	Drops the chargemaster’s role in turning services into charges.

7. Answer C Concept 6

A physician practice ~~with 14 providers and a new billing vendor~~ has \$1.2 million in net accounts receivable. Its net patient revenue for the past 365 days was \$10.95 million. What is days in accounts receivable?

Target word: days in A/R **Decides:** Daily revenue = $\$10.95\text{M} / 365 = \$30,000$.

A	Dropped element	Divides A/R by annual revenue without converting to days.
B	Inverted operation	Divides revenue by A/R.
C	Correct	$\$1,200,000 / \$30,000 = 40$ days.
D	Wrong stem number	That is the period length.

8. Answer A Concept 6 • transfer

A hospital’s goal is 45 days in A/R. Its average daily net patient revenue is \$200,000. What is the highest net A/R balance that meets the goal?

Target word: solve for A/R **Decides:** $\text{A/R} = \text{days} \times \text{daily revenue}$.

A	Correct	$45 \times \$200,000 = \$9,000,000$.
B	Confusion set	That is a year of revenue (365 days).
C	Inverted operation	Divides daily revenue by days.
D	Absolute language	The goal allows up to 45 days of revenue in A/R.

9. Answer D Concept 7

A 67-year-old engineer who still works full-time at a company with 300 employees has Medicare Part A and coverage through his employer's group health plan. ~~He recently completed a marathon.~~ He is admitted for a hip repair. Which payer is primary?

Target word: coordination of benefits **Decides:** Current employment coverage at an employer with 20+ employees.

A	Adjacent category	Coordination of benefits sets an order, not a split.
B	Confusion set	Which Part applies does not decide payer order.
C	Inverted operation	Reverses the order for someone covered through current employment.
D	Correct	Medicare is secondary for working-aged beneficiaries covered through a 20+ employee employer.

10. Answer A Concept 7 • transfer

A 72-year-old retiree has Medicare and a retiree health plan from her former employer. Which pays first?

Target word: payer order — retired **Decides:** Retiree coverage is not current employment coverage.

A	Correct	Retiree plans pay after Medicare.
B	Inverted operation	Applies the working-aged rule to a retiree.
C	Adjacent category	Deductible size does not set payer order.
D	Absolute language	Only current-employment coverage (with size rules) pays before Medicare.

11. Answer C Concept 8

A primary care group ~~that recently moved to a larger building~~ accepts a capitated contract covering 2,000 members at \$30 per member per month. ~~Last year its physicians averaged 22 patient visits per day.~~ What is the group's annual revenue from this contract?

Target word: annual capitation revenue **Decides:** Members × PMPM × 12.

A	Confusion set	That is fee-for-service logic; capitation is fixed per member.
B	Dropped element	One month's payment; drops the × 12.
C	Correct	$2,000 \times \$30 \times 12 = \$720,000$.
D	Inverted operation	Divides members by the rate.

12. Answer D Concept 8 • transfer

Under a capitated contract, a provider group reduces unnecessary follow-up visits by 10% while membership stays the same. What happens to its capitation revenue?

Target word: revenue effect of lower utilization **Decides:** Payment is per member, not per visit.

A	Confusion set	Applies fee-for-service logic.
B	Inverted operation	Revenue does not move with visit counts in either direction.
C	Absolute language	Capitation rewards efficient use.
D	Correct	Capitation revenue depends on members, so fewer services raise profit.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	6	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	5	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	10	
Absolute language	Always, never, guarantee — where judgment applies.	5	
Confusion set	Every option is real; the test is exact role-matching.	6	
Camouflage	Answers a stem fact that was never the question.	3	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	MS-DRG payment	Medicare inpatient payment = MS-DRG relative weight × hospital base rate, a fixed amount per case regardless of length of stay (outliers aside).	Scaling the payment up for a longer stay.	A relative weight, a base rate, and a length of stay.
2	Which Medicare payment system	Inpatient hospital = IPPS/MS-DRG; hospital outpatient = OPPOS/APC; physician professional services = Physician Fee Schedule (RBRVS).	Paying a hospital outpatient facility fee under DRGs or the physician fee schedule.	The setting and whether it is a facility or professional claim.
3	RBRVS fee calculation	Fee = geographically adjusted total RVUs × conversion factor.	Dividing by the conversion factor or multiplying by annual volume.	RVUs and a dollar conversion factor.
4	Front-end revenue cycle	Eligibility, authorization, and registration accuracy prevent most denials; fix denials at their root cause, often the front end.	Treating denials as a back-end appeals problem.	A denial reason like “not eligible” or “no authorization.”
5	Chargemaster and charge capture	The chargemaster links each service to a charge code, revenue code, CPT/HCPCS code, and price; a missing line means the service is not billed.	Assuming clinical documentation alone produces a charge.	A new service not added to the chargemaster.
6	Days in accounts receivable	Days in A/R = net A/R / (net patient revenue / 365); lower is better.	Inverting the ratio or leaving out the daily conversion.	A receivable balance and annual revenue.
7	Medicare secondary payer	For a working person 65 or older covered through current employment at an employer with 20 or more employees, the group health plan pays first; retiree coverage pays after Medicare.	Assuming Medicare is always primary at 65.	Age 65+ plus “still working” vs. “retired.”
8	Capitation	Capitation pays a fixed amount per member per month regardless of services used, shifting utilization risk to the provider.	Thinking revenue rises or falls with visit volume, as in fee-for-service.	“Per member per month.”

Course scope (public description): Healthcare Reimbursement explores financial practices within the healthcare industry as they relate to reimbursement policies. This course identifies how reimbursement systems impact the revenue cycle and a health information manager's role. This course has no prerequisites.